

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°140/MVE-BDBV/01/10/2026

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HIGHLIGHTS (last 24 hours)

Seventy-six new confirmed cases of Ebola Virus Disease (EVD) have been reported, including 19 community deaths. These cases originated from the provinces of Ituri (49 cases), North Kivu (23 cases), Haut-Uélé (3 cases), and Tshopo (1 case). In addition, 5 deaths were recorded in Ebola Treatment Centers (ETCs).

A total of 452 new samples were analyzed; 76 of these samples were found to be positive for the Bundibugyo virus, representing a positivity rate of 16.8%.

No new health zones have reported any confirmed cases in the last 24 hours.

ACCUMULATIVE
CAS
8 376

ACCUMULATIVE
DEATH
CONFIRMED
4 042

LETHALITY
48,3 %

PATIENTS IN
ISOLATION/CTE
869

ACCUMULATIVE
HEALED
2 182

TRACKING RATES
CONTACTS
(Of the Day)
81,3 %



Affected provinces and affected health zones



7 Provinces affected

Uele, Ituri, North Kivu,
South Kivu, Tshopo, Lower Uele and
Sud Ubangi



63 out of 167
health zones affected
(37,7%)

Summary of key points from the last 24 hours

Over the past 24 hours, 76 new confirmed cases of Ebola Virus Disease (EVD), including 19 community deaths, have been reported in 4 provinces. The provinces that reported these cases are Ituri (49 cases), North Kivu (23 cases), Haut-Uélé (3 cases) and Tshopo (1 case).

In addition, 5 deaths were recorded in the CTEs (4 in Ituri and 1 in North Kivu), bringing the total number of confirmed deaths for the period under consideration to 24.

However, 20 patients (19 in Ituri, 1 in North Kivu) were declared cured after obtaining two negative results for Ebola virus disease.

As of October 1, 2026, the DRC has recorded 8,376 confirmed cases and 4,042 deaths.

(Case fatality rate: 48.3 %). Ituri remains the epicenter of the epidemic with 75.7% of cumulative confirmed cases reported and 64.5% of new confirmed cases. In contrast, North Kivu province has a case fatality rate higher than the national average, at 59.0%.

In total, 63 out of 167 health zones (37.7%), spread across seven provinces, have been affected since the start of the epidemic.

Table 1. Distribution of confirmed cases and deaths by affected province, as of October 1, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	49	6342	2 918	46,0%	28/36 (77,8 %)
North Kivu	23	1624	958	59,0%	16/34 (47,1 %)
Haut-Uélé	3	349	144	41,3%	7/13 (53,8 %)
Boss	1	46	16	34,8%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	10	4	40,0%	3/11 (27,3%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Total	76	8 376	4 042	48,3%	63/167 (37,7 %)

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 1, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	6342	2918	46,0%	49	12	4	16
Give	11	1	9,1%				0
Underwood	8	6	75,0%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	202	46	22,8%				0
have	2	2	100,0%				0
Bunia	1754	572	32,6%	15	5	1	6
Ladies	40	9	22,5%				0
Drodoro	15	7	46,7%				0
Fireworks	82	31	37,8%				0
Gethy	7	3	42,9%				0
Flounder	2	0	0,0%				0
Kilo	37	12	32,4%				0
Team	170	104	61,2%	1	1		1
Liter	288	145	50,3%	1	0	0	0
Logo	13	5	38,5%				0
Fight	34	9	26,5%				0
distributed	4	1	25,0%				0
Washing	32	12	37,5%				1
Darkness	152	79	52,0%	14	4	0	4
Mangala	362	156	43,1%	5	2	0	2
Mongbwalu	705	333	47,2%	2	0	0	0
Nanny	305	167	54,8%				0

Nizi	783	280	35,8%	2	0	1	1
Legumes	129	37	28,7%				0
Jungle	12	4	33,3%				0
Rwampara	1097	372	33,9%	9		1	1
Tchomia	76	25	32,9%				0
A valves	AN	493	THAT				0
North Kivu	1624	958	59,0%	23	7	1	8
Me	367	243	66,2%	6	4		4
Farm	19	7	36,8%				0
Butembo	270	173	64,1%	3	1	1	2
Ten	1	0	0,0%				0
Kalunguta	57	19	33,3%	1	0		0
Cut	571	359	62,9%	5			0
Source	1	0	0,0%				0
Kyondo	33	20	60,6%				0
Lubero	21	11	52,4%				0
Be careful.	38	14	36,8%	2	0		0
Manguredjipa	13	5	38,5%				0
Masereka	31	10	32,3%	2	0		0
The museums	126	68	54,0%				0
Mutwanga	15	3	20,0%	1	1		1
Oicha	29	16	55,2%	2	1		1
Vuh	32	10	31,3%	1			0
Upper Uele	349	144	41,3%	3	0	0	0
Government							
Mangbetu	41	17	41,5%	2	0	0	0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	102	44	43,1%				0
Lining	87	39	44,8%				0
Beans	3	1	33,3%	1	0	0	0
Ordinary	112	41	36,6%				0
Boss	46	16	34,8%	1	0	0	0
Bafwasende	7	1	14,3%				0
Kabondo	5	1	20,0%				0
Lubunga	1	0	0,0%				0
Maquis - Kisangani	25	10	40,0%	1	0	0	0
Mangobo	6	3	50,0%				0
Boss	1	0	0,0%				0
Wanie - Arugula	1	1	100,0%				0
South Kivu	3	1	33,3%	0	0	0	0
Mili -Murhesa	3	1	33,3%				0
Bas Uélé	10	4	40,0%	0	0	0	0
Blind	1	1	100,0%				0

Viadana	6	1	16,7%				0
Walk	3	2	66,7%				0
Sud Ubangi	2	1	50,0%	0	0	0	0
Hair	2	1	50,0%				0
Total	8376	4042	48,3%	76	19	5	24

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.
NA : Not Applicable.

Table 3. Status of alerts notified by province for October 1, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	39	4	43	27	4	12	0	31	18
Bas Uélé	157	4	161	29	3	129	0	32	0
Ituri	927	81	1008	176	79	743	1	255	137
North Kivu	1 041	39	1080	140	39	862	0	179	100
Sud Ubangi	56	1	57	3	1	53	0	4	0
Boss	91	2	92	11	2	79	0	13	0
Total General	2 311	91	2 441 386		128	1 878	1	514	255

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

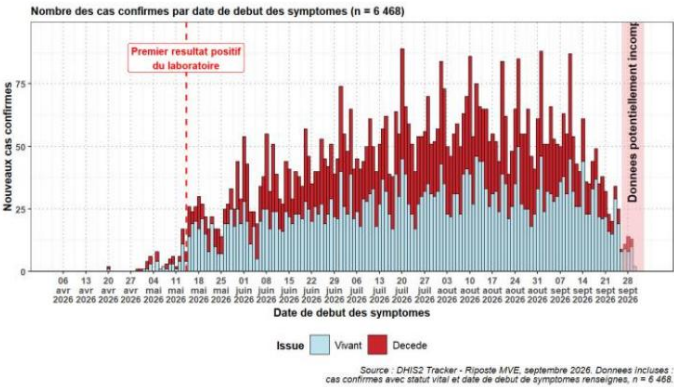
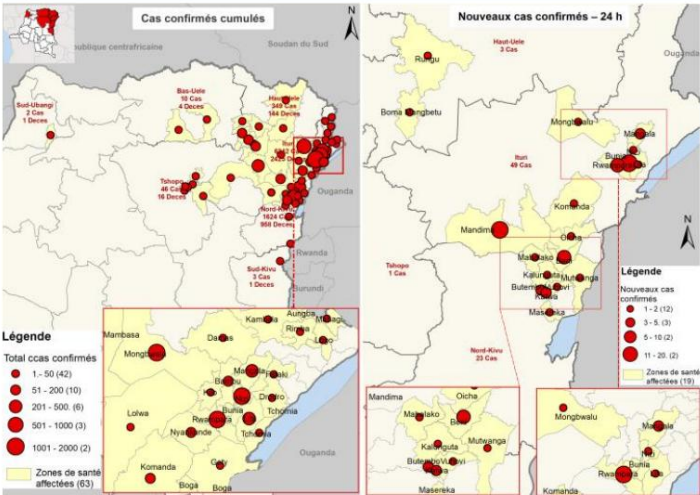


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

By the end of the day on October 1, 2026, **2,402 alerts** had been received, **2,393** (99.6%) of which were verified, including **514** (21.5%) validated as suspected cases. Of these, **514** (100%) were investigated. Consequently, 255 suspected cases were transferred to the CT/CTE ([Table 3](#)).

Of the **31,092** contacts currently being monitored, **25,277** were seen in the last 24 hours, representing a follow-up rate of **81.3 %**, slightly higher than the previous day's rate (80.6%) but still below the required threshold (95.0%). This rate was 90.3% (167/185) in Bas-Uélé, 87.2% (34/39) in Sud-Ubangi, 87.1% (660/758) in Tshopo, 84.9% (882/1,039) in Haut-Uélé, 83.4% (12,169/14,591) in Ituri, and 78.5% (11,365/14,480) in Nord-Kivu.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

Seven (7) alerts were notified to the PoE/PoC during the day, including six (6) in Ituri and one (1) in North Kivu. In Ituri, at the Epulu Zunguluka PoC (Mambasa Health Zone), four alerts were validated: three suspected cases were transferred to the Mambasa General Referral Hospital's Treatment Center (CTE), and the fourth chose to continue treatment in Mangina (transfer rate of 75.0%); at the Dele PoC (Rwampara Health Zone), a body was intercepted and taken to the morgue for swab testing, and the Mabakese PoC alert was invalidated after secondary screening. In North Kivu, the Maboya PoC alert (Kalunguta Health Zone) was validated, and the patient was referred to the Tinge Hospital.

Table 4. Status of key indicators for PoE/PoC by province, as of October 1, 2026

Indicators	Ituri	North Kivu	Boss	Global
Report completeness	98,3% (59/60)	95,2% (20/21)	100,0% (18/18)	98,0% (97/99)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	152 190	100 612	22 301	275 103
% of travelers screened	97,9%	97,8%	98,5%	97,9%
% of travelers who washed their hands	97,5%	97,8%	98,0%	97,6%
% of travelers made aware	99,5%	97,8%	100,0%	98,9%
Number of alerts notified	6	1	0	7
Number of validated live alerts (suspected cases)	4	1	0	5
Number of bodies recovered today	1	0	0	1
% of suspected cases transferred to CTE/CT	75,0%	100,0%	THAT	80,0%
% of lifeless bodies swabbed/secured	100,0%	THAT	THAT	100,0%
% of suspected cases investigated within 4 h	100,0%	100,0%	THAT	100,0%

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 49 new positive results** (37 living and 12 dead) out of 291 samples received and analyzed (**positivity: 16.8%**).
- **North Kivu: 23 new positive results** (16 living and 7 dead) out of 142 samples received and analyzed (**positivity: 16.2%**).
- **Haut-Uélé: 3 new positive results** (3 living and 0 deaths) out of 8 samples received and analyzed (**positivity: 37.5%**).
- **Tshopo: 1 new positive result received** (1 live sample), out of 11 samples received and analyzed (positivity: 9.1%).
- **Bas Uélé: 18 samples** were taken and sent to the Isiro laboratory; results are pending waiting.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 18 out of 29 rings were opened (62.1%), 37 out of 38 households were decontaminated (97.4%), and 5 out of 5 health facilities were decontaminated (100%). Of 92 EDS alerts (including 4 postponements), 57 EDS were carried out (62.0%) and 63 bodies were swabbed; no households, health establishments, or schools received infection control kits.
- **North Kivu:** 16 out of 18 treatment rings were opened (88.9%), 10 out of 24 households were decontaminated (41.7%), and 11 out of 13 community health centers (84.6%) were decontaminated. In total, 334 community health centers were monitored and 393 service providers were briefed; no decontaminated household received a kit.
- **Haut-Uélé :** 2 out of 2 rings were opened (100%), 9 out of 9 households were decontaminated (100%), and 2 out of 2 ESSs were decontaminated (100%). Out of 6 EDS alerts, 5 EDSs were carried out (83.3%), and 5 bodies were swabbed in Isiro and Wamba; UNICEF provided the Isiro health zone with 2 vehicles and a decontamination kit.
- **Tshopo :** 16 health centers were monitored in 6 health zones and 173 service providers were briefed (100%); integrated training of 150 private health center staff continues in Makiso-Kisangani. Cumulatively, 36 out of 36 health centers (100%) and 27 out of 31 households (87.1%) that hosted a confirmed case were decontaminated within 48 hours. Four out of six bodies admitted to morgues were swabbed, without a complete blood count (CBC).
- **Bas-Uélé :** 8 out of 9 ESS (88.9%) were monitored in Viadana, Buta, and Ganga, and 29 out of 30 service providers were briefed (96.7%), with 3 out of 4 triage areas (75.0%) operational in Buta. Of the 4 death alerts, 3 bodies were swabbed, without an EDS (Emergency Data System) being performed.

2.4.2. Challenges

In **Ituri**, the opening of the rings remains insufficient (62.1%; 18/29), 27 bodies have not been swabbed due to a lack of supplies and teams in Mandima or following refusals by families, resistance persists in Nizi, and no household, social economy, or school has received kits. In **North Kivu**, Household decontamination remains low (41.7%; 10/24) due to community resistance and a lack of teams and mobility. In **Tshopo**, the shortage of infection prevention and control (IPC) supplies in health centers and services (HSS) and the breakdown of the incinerator at Makiso-Kisangani General Referral Hospital (HGR) persist. In **Bas-Uélé**, the Triangle Health Center lacks a functional waste area and hygienic latrines, and strong resistance to swab treatment continues in the Bomea Health District (Buta Health Zone).

2.5. Holistic care

2.5.1. Main actions

- In **Ituri**, 100 new admissions were recorded, compared to 108 discharges, including 19 recoveries. A total of 394 patients remained in isolation at the end of the day. The bed occupancy rate was **38.8%** ; 346 out of 375 patients (92.3%) received hot meals, and children under LNPE/SLM were fully covered at the CTE (13/13), in daycare (5/5), and in the community (50/50); 27 patients are in critical condition.
- In **North Kivu**, 71 new admissions were recorded, compared to 57 discharges (1 recovery). A total of 383 patients remained hospitalized, including 271 in standardized facilities. The bed occupancy rate was **63.0%** ; 21 patients were in critical condition at the Beni Treatment Center.
- In **Tshopo**, 2 new admissions (1 confirmed and 1 suspected) were recorded. None No discharge was reported. Twelve (12) patients remained in isolation at the end of the day, representing an occupancy rate of 38.7%.
- In **Bas-Uélé**, 2 new admissions of suspected cases were recorded at the Limba CT (Viadana Health Zone) compared to 3 discharges of suspected cases who tested negative in Buta. Six (6) patients remained in isolation at the end of the day.
- In **Haut-Uélé**, 11 new admissions were recorded. In total, 74 patients are in isolation in CTEs, i.e. an occupancy rate of **53.6 %**.

2.5.2. Challenges

- In **Ituri**, insufficient capacity of some CTE/CT facilities and inadequate care food-related issues remain a concern.
- In **Bas-Uélé**, the lack of a standardized CTE and ambulance for the transfer of patients remains a major operational challenge.

- In **Tshopo**, the continued presence of confirmed cases in isolation necessitates the maintenance of infection prevention and control measures.

2.6. Vaccination

2.6.1. Main actions

- In **Bas-Uélé**, 25 PPL and TPL were vaccinated in Buta and compassionate vaccination started in the Aketi health zone (11 vaccinated), bringing the provincial total to 1,071 people vaccinated out of a target of 13,202 (8.1%), mainly in Buta (610; 21.3%) and in Ganga (273; 20.5%).

2.6.2. Challenges

In **Ituri**, the target for PPL/TPL remains to be redefined, which prevents the calculation of a reliable coverage rate.

In **Bas-Uélé**, the UCCs have not been put into operation, the validated micro-plan and vaccination funds are not available.

2.7. Continuity of Care

2.7.1. Main actions • In Tshopo, the

evaluation of the Zawadi Health Center noted the absence of certain registers (CPS and CPN), staff vaccination limited to 3 out of 9 agents (33.3%) and a non-effective triage area; the recovery plan for the Céleste Polyclinic was developed.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- In **Ituri**, 22,619 home visits reached 56,036 people; 460 educational talks brought together 7,760 people, 50 community debates reached 923 people, and 75 mass awareness campaigns reached 5,119 people. A total of 259 community alerts, including 59 deaths, were reported; 58 out of 90 feedback requests (64.4%) and 8 out of 9 incidents (88.9%) were resolved within 48 hours (9 out of 28 health zones completed, or 32.1%).
- In **North Kivu**, 13,038 home visits reached 45,515 people, 89 educational talks reached 4,479 people and 38 mass communication sessions reached 7,648 people; 256 out of 650 expected community alerts (39.4%) were notified, and 17 out of 23 feedbacks (73.9%) as well as the 5 recorded incidents were resolved within 48 hours.
- In **Tshopo**, 198 home visits reached 1,410 people, 2 educational talks reached 161 people and 8 mass awareness sessions reached 4,377 people; 13 community alerts were notified (8 in Bafwasende, 3 in Kabondo and 2 in Tshopo) and the 2 feedbacks received were resolved (2/2).
- In **Bas-Uélé**, 200 home visits reached 1,275 people, 8 educational talks reached 123 people and 10 mass awareness campaigns reached 1,800 people in Viadana, Titule and Ganga; the 9 community alerts notified in Ganga were dealt with).

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- In **North Kivu**, 25 out of 31 confirmed cases (80.6%) recorded the previous day and 246 confirmed cases under follow-up benefited from SMSPS interventions in 7 out of 8 health zones that reported (87.5%). The teams took charge of 112 new suspected cases and 406 suspected cases under follow-up, provided support to 34 households of separated and orphaned children (100%), announced 108 results, including 12 positive ones, and reintegrated 8 recovered individuals.
- In **Ituri**, 34 out of 44 confirmed cases (77.3%) from the previous day, 126 confirmed cases under observation out of 133 (94.7%), 57 new suspected cases out of 100 (57.0%), and 122 suspected cases under observation out of 133 (91.7%) benefited from SMSPS interventions. The teams supported 447 caregivers and 78 people living with HIV and reported 56 results, including 34 positive ones.
- In **Tshopo**, the teams provided psychological support to 3 confirmed cases at the CTE Météo, including 1 new case, carried out psychoeducation for 10 suspects and 196 people, including 117 trainee doctors at CUKIS, and announced 3 results, including 1 positive one.

2.9.2. Challenges

- In Ituri, 13 out of 28 affected health zones are covered (46.4%); coverage around the latest cases remains low in Nia-Nia (2/7; 28.6%), where SMSPS interventions around community deaths need to be strengthened.
- In Haut-Uélé, difficulties in the mobility of APS in Wamba and Boma Mangbetu, insufficient nutritional kits for weaned infants and poor data reporting in Pawa and Boma Mangbetu.
- In Tshopo, there is a need to train the APS (Auxiliary Security Personnel) of the ZS (Health Zones) of Wanie-Rukula and Bafwasende, and insufficient means of communication and movement of the pillar.

2.10. Logistics

2.10.1. Main actions

- At Tshopo, receipt of 2 rental vehicles for 42 days (MOMENTUM), followed by the construction of the Kongakonga isolation, the Bafwasende CTE and the extension of the Météo CTE.
- In Bas-Uélé, deployment of surveillance teams in Ganga and Viadana, continuation of the rehabilitation of the meeting room and offices of the DPS (WHO) and of the construction of the transit center of the HGR of Buta; the stock of chlorine is limited to 25 kg and diesel is out of stock.

2.11. Security

2.11.1. Main actions

- In North Kivu, the Risk Mitigation and Security pillar carried out 2 security coverage activities (100%), including accompanying the EDS team for the burial of a confirmed case in Bundji, and restoring order at the morgue of the Beni General Referral Hospital, where a family was trying to forcibly recover the body of a confirmed case).

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

- In Ituri, 24 staff members (17 men and 7 women) were briefed on the PSEAH and signed the code of conduct in Bunia; only 13 out of 36 health zones (36.1%) know the reporting mechanism.
- In Haut-Uélé, 29 PPLs from the Mayogo AS (Isiro Health Zone), including 19 women, were briefed on the PSEAH and 63 people, including 36 pregnant women received in CPN at the VDO Health Center, were sensitized in Wamba and Isiro.

2.12.2. Challenges

At the national level, only 2 DPS reported their PSEA activities (28.6%) and CBCMs remain largely ineffective.

MESSAGES CLES	TIMELINE OF KEY EVENTS
↻ Active and sustained community transmission remains observed in the provinces of Ituri, North Kivu, and Haut-Uélé. Meanwhile, the province of Sud-Ubangi has not reported any confirmed cases in the last 24 hours, and a lull has been observed in the province of Tshopo compared to the last 72 hours. ↻ Progress has been made in epidemiological investigations: Almost all (100 %) suspected cases were investigated within 24 hours of notification. However, the contact tracing rate is higher than the previous day (81.3% vs. 80.6%). Nevertheless, efforts must be made to bring this rate up to the required threshold (95%). ↻ Strengthen surveillance, care, IPC, logistics and community engagement, prioritizing increased capacity of ETC/CT in North Kivu, rapid transfer of suspects and isolation of confirmed cases. ↻ Strengthen preparedness and response to Ebola Virus Disease (EVD) in the river corridor and surrounding areas, particularly in South Ubangi, through active case finding, contact tracing, and ensuring the availability of supplies. Continue cross-border surveillance with South Sudan.	↻ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo. ↻ Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.
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